Health

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Summary

For a long time, health was simply defined as the absence of disease, equating to the absence of physical and mental dysfunction. A paradigm shift took place with the World Health Organization's (WHO) definition of health in 1948. Health was now defined positively as 'a state of complete physical, mental and social well-being'. This definition has three key features: a positive view of health; a move away from narrow biomedical perspectives and healthcare systems; and a multidimensional approach. However, it also presents major problems in terms of operationalisation and implementation. More recent, interdisciplinary definitions emphasise balance, biographical adaptation and productive processing of reality, as well as coping in interaction with environments. Relative and functional health (according to the ICF) are also important aspects of a realistic yet complex definition of health. In future, health must be strengthened and expanded as a 'strong' reference category in its own right, both scientifically and in terms of healthcare policy.

Key words

Health, classifications, WHO definition, multidimensionality, interdisciplinary definitions of health, model of productive processing of internal and external reality, functional health, ICF, health promotion, health as a reference category

General conceptualisation and early definition attempts

Health is an elusive concept that cannot be clearly defined. Like disease and illness, it is produced and organised both individually and socially. It is a relative and relational phenomenon influenced by cultural, socio-political and ecological contexts, and it is constantly evolving (Nettleton, 2020; Richter & Hurrelmann, 2023).

Earlier attempts at defining and classifying health were almost always based on an opposition to the concept of illness. Seedhouse (2001) identified four core scientific concepts of health:

- An ideal state of complete well-being without any physical, mental or social disorders.
- Personal strength based on physical and mental characteristics.
- The ability to fulfil social requirements, especially everyday role obligations.
- A commodity that can be manufactured and ‘purchased’.

Although these concepts view health from different perspectives, they cannot yet be combined to form a comprehensive definition. To date, there is still no universally accepted definition of health within the interdisciplinary field of practice and research.

In the early 1990s, the German medical sociologist Göckenjan proposed three interpretative approaches.

Health as a defining or 'demarcation' conception: This is closely linked to the biomedical interpretation and diagnosis of disease. Health is described as the absence of illness, or as not being ill yet, or as a potentially harmful physical or mental condition that has not yet been fully diagnosed medically. This distinction is often necessary for legal purposes, such as obtaining a certificate of incapacity for work or reduced earning capacity.

Health as a functional statement: Health is defined as adequate physical and social performance, working capacity, and the fulfilment of roles (with a sociological focus

on health and illness). This includes homeostatic concepts of health based on physical and mental balance, as well as the body's and self's flexible adaptation to changing environmental conditions. The emphasis is often placed on people's resilience to pathogens, infections and/or life-threatening experiences. Function-oriented descriptions can also be interpreted as metaphors.

Health as a normative, value-oriented concept: Health is regarded as a supreme value and an absolute standard. Some of these approaches also attempt to justify normative health constraints and the obligation to optimise.

In recent international debates, proposals have been put forward that emphasise the contextual nature of health definitions. A distinction can be made between general, overarching health concepts, such as the WHO definition, and various operational definitions. The latter refer to a precisely defined context. These are used for evidence-based measurement, documentation, and tracking progress.

The WHO Definition

The preamble to the 1948 Constitution of the World Health Organization (WHO) provides the most well-known value-oriented description of health promotion and is particularly relevant in this context. It states that ‘Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity. The enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being without distinction of race, religion, political belief, economic or social condition.‘ (WHO, 2020, p. 1). These statements are known worldwide as the 'WHO health definition'.

The WHO has moved away from a purely biomedical perspective and the narrow confines of professional, disease-oriented care systems. Health is defined positively and determined multidimensionally – this is what ‘complete‘ stands for. Health encompasses the physical, mental, and social aspects which influence each other.

The WHO's definition also inspired the biopsychosocial model of illness and health, which reached scientific maturity three decades later (Engel, 1977). Subsequently, the WHO applied the principle of multidimensionality to mental health as well.

Although the preamble text is often referred to, health should not be considered a static state that is achieved once and then remains unchanged. Rather, health should be understood as a dynamic process and a potential that is constantly readjusted throughout life and in everyday situations, requiring active maintenance of a balance between resources and stresses.

Unlike the biomedical-naturalistic understanding of illness, the WHO's concept of health emphasises the importance and anchoring of well-being in all aspects of daily life. Health-conscious and health-promoting lifestyles require positive political, cultural, economic, and socio-ecological conditions — in other words, supportive and empowering determinants of health. The highest attainable standard of health is declared to be an indisputable human right. Its preservation and protection is the responsibility of both nation states and global society.

In the 1980s, the spiritual dimension was added to the final documents of key WHO assemblies as a fourth pillar of well-being. However, there were no corresponding revisions to the 1948 constitutional text. The discussion about spirituality as a fourth dimension remains important and influential in the Asia-Pacific region and among Arab-Islamic WHO member states. In Europe and North America, however, this issue is largely ignored.

Critical perspectives, challenges and developments

The WHO definition has been widely criticised. The main criticisms are that it is one-sided and subjective; that it is imprecise and difficult to operationalise or measure; and that its thinking on poles/extremes is still rather static and

immature. The definition is often accused of being dogmatic and of setting an unattainable standard. However, this overlooks its historical and political context, and the fact that it was formulated in the aftermath of the devastating Second World War (Schramme, 2023).

The concept of the 'state' and the idea of 'complete' well-being have both been the focus of much criticism, with the latter being dismissed as an absolute utopia. Kickbusch (1999), one of the main architects of the 1986 Ottawa Charter, interprets the pursuit of 'comprehensive well-being' as a universal, positive utopian imperative. This should, however, be understood as an ethical imperative to establish and secure health for all, in line with every state's obligation to provide equal health opportunities within a community and nation.

Against the backdrop of multifaceted global transformations and transitions since the mid-20th century, limitations due to epidemiological restrictions have become apparent. Three trends are particularly significant:

- The global changes in population demographics brings with it challenges relating to health security and care for societies with long life expectancies, which are often coupled with chronic limitations and lifelong treatment needs in older subpopulations.
- A profound upheaval in risk and disease patterns is leading to global changes in the landscape of health problems and an expansion of systemic professional detection, treatment and support.
- Social and environmental determinants of health and all associated inequalities and gradients are becoming increasingly significant.

The limitations of the WHO definition of health, particularly with regard to its application in clinical medicine and public health research, cannot be overlooked. Its strength lies in ‘encouraging consideration of the determinants of health — the factors that make people sick or healthy — and the mechanisms at work in this process. The WHO definition can be understood as a health policy vision or an ideal to be striven for by all‘ (Razum & Kolip, 2020, p. 21). Health, as defined by the WHO, has never been considered a measurable quantity, but rather a 'normative good'. It was never intended for use in efficiency measurements, nor is it suitable for this purpose.

Newer health concepts and definitions can transcend the WHO definition's limitations by explicitly recognising that people with chronic illnesses can also be healthy. The emphasis is then no longer on 'complete' health, which is often misinterpreted as the unattainable and exclusionary concept of 'perfect' health. Instead, the focus is on all the skills and resources required to actively and productively overcome life's inevitable challenges (see below for an interdisciplinary definition of functional health).

Despite the justified criticism it has received, the WHO definition continues to be influential in policy areas such as prevention and health promotion (Wills, 2022; Hurrelmann et al., 2024), and still incorporates important integrative aspects. It also remains conceptually valuable in the context of medical and health sciences.

Integrative and interdisciplinary definitions of health

Since the late 1980s, key scientific theories have informed the development of a scientifically robust concept of health in German health discourse. Theories relating to biomedicine, learning, personality, stress and coping mechanisms, social interactions and structures, and public health are considered of equal importance.

Hurrelmann has integrated these theories into an interdisciplinary 'model of productive processing of internal and external reality' (Hurrelmann, 1989; Hurrelmann & Richter, 2020, p. 69), which is derived from socialisation theory.

Against this background, eight interdisciplinary principles for defining health and illness were derived (Hurrelmann & Richter, 2020, pp. 69-82):

1 Health and illness are the result of an interplay between social and personal circumstances that influence and shape health behaviour.
2 Social conditions determine the scope of opportunities for developing personal health and illness conditions.
3 Health is a state of equilibrium, whereas illness is a state of imbalance between risk and protective factors at physical, psychological and social levels.
4 Health and illness are the respective endpoints of equilibrium and imbalance with physical, psychological, and social dimensions.
5 Health is the result of successfully coping with internal and external demands, whereas illness is the result of unsuccessful coping.
6 The personal prerequisite for health is a lifestyle that is body-conscious, psychologically sensitive and environmentally oriented.
7 The characteristics and stages of health and illness are subject to subjective appraisal.
8 External and self-assessment of these stages can differ in all three dimensions: physical, psychological, and social.

Individual capacities are the fundamental means by which external demands must be met. These include socio-economic factors, the ecological environment, living conditions, hygiene, educational opportunities, working conditions, personal habits, and social integration. In order to successfully meet internal requirements and manage the internal reality of their body and psyche as productively as the external reality of their social and physical environment, a person must have these resources at their disposal.

Hurrelmann and Richter derive the following definition of health and illness from these considerations: ‘Health is the equilibrium of risk and protective factors that occurs when a person is successful in coping with internal (physical and psychological) and external (social and material) requirements. Health is a stage that conveys a sense of well-being and enjoyment of life to a person.

Illness is the imbalance of risk and protective factors, which occurs when a person is unsuccessful in coping with internal (physical and psychological) and external (social and material) requirements. Illness is a stage, which impairs or adversely affects the well-being and life enjoyment of a person.

We can speak of relative health or relative illness to convey that there can also be intermediate positions of temporary duration between these two absolute stages of health and illness, which must be continuously reproduced in every life stage and situation.’ (Hurrelmann & Richter, 2020, pp. 75-76).

The most important elements of a person’s healthy lifestyle are having a positive attitude towards everyday challenges, accepting one's body and personality — including any temporary or permanent impairments or limitations — and maintaining a sense of optimism about the future. It also involves having optimistic expectations of the social environment and the belief that one's lifestyle can be influenced. This concept is referred to as 'self-efficacy' in theories of learning and development, 'sense of coherence' in salutogenic theory and ‘productive processing of internal and external reality' in socialisation theory. It is important to combine self-confidence with a conscious lifestyle that focuses on work, performance, stress management, rational thinking, relaxation, enjoying wholesome food and drink, exercising, forming attachments, and experiencing love and a fulfilling sex life.

Related definitions and connections

Other researchers have also provided definitions focusing on biographical adaptation, balance, and coping in the context of perpetually changing and complex life and environmental challenges.

Huber et al. (2011; 2016) view health as the dynamic ability to cope with and manage

social, physical and emotional challenges in a self-determined way throughout one's life.

Bircher (2020) defines health as a dynamic state of well-being, as well as a complex adaptive system that enables individuals to cope with the demands of life.

Krahn et al. (2021) define health as the dynamic interplay of physical, mental, social and existential well-being when adapting to and coping with inevitable life conditions and the environment.

Further conceptual parallels and connections can be found in systemic approaches to defining health (Hafen, 2016) and in theoretical medicine's Holistic Model of Health (Nordenfelt, 2018). According to the HMH, health is a function of all personal abilities and competencies. It is a prerequisite for carrying out intentional actions and achieving one's vital goals within the respective cultural framework and through social exchange.

This perspective establishes links with Nussbaum and Sen's concept of participation and realisation opportunities – the human capabilities approach –, particularly with regard to health promotion. This issue has been highlighted in a pioneering way in the German-speaking world by Abel (Abel & Frohlich, 2012; Frahsa et al., 2020)). Lastly, the basic assumptions of the HMH fully accommodate both limited health and chronic illness, rather than excluding them.

The socio-epidemiological notion of ‘social health‘ (Paul et al., 2023) is characterised by resource availability and social co-determination. Health is viewed as the result of accumulating individual and collective resources, including crisis management. Health arises (and is maintained) through successful exchange of resources between individuals and society. Conversely, illness is considered to be the result of resource crises. Both the accumulation of coping resources and resource crises are socially influenced and controlled.

Therefore, health is always the result of collective action. The concept of social health indicates whether individuals and groups are able to accumulate and secure health-enhancing resources, and ensure their fair distribution.

Relative, conditional and functional health

Health cannot simply be optimal, comprehensive or positive. Any concept of health must acknowledge its potential limitations. People coping with acute disorders or living with chronic physical and mental health conditions are not solely defined by their illness. They almost always have abilities that benefit their health when dealing with their problems and illnesses, and when organising their everyday lives. They also have a variety of useful skills and resources at their disposal. As such, they experience relative or conditional health.

Relative health is a dynamic state in which the balance of risk and protective factors is disturbed to some extent. This occurs when a person is only partially or temporarily able to cope with internal and external demands. The similar term 'conditional health' has long been used in German-speaking rehabilitation. It describes the ability to actively shape one's life and perform well at work and in everyday life despite having a chronic illness. People with a chronic illness or permanent physical or mental impairment are 'conditionally healthy' if they can achieve a dynamic balance that enables them to lead fulfilling lives and reach their goals within certain limits.

The technical term that dominates in this context internationally is 'functional health'. It is the core concept of the International Classification of Functioning, Disability and Health (ICF) and plays a pivotal role in the fields of rehabilitation medicine, epidemiology and health monitoring. A person is considered functionally healthy if, given their contextual factors,

- Their physical functions, including mental functions and body structures, correspond to those of a healthy person (body functions and body structures);

- They do or are able to do everything expected of a person without health problems (the activities concept).
- They are also able to develop their life in all areas that are important to them, in the manner and to the extent expected of a person without health-related impairments of bodily functions, body structures, or activities (participation in life).

Due to its foundation in a biopsychosocial model, the ICF is not primarily deficit-oriented. It focuses on three interacting components: resource orientation, aetiology neutrality, and the concept of a continuum. The ICF can be applied universally, i.e. to all people, not just those with disabilities or limitations.

Health as a 'strong' reference category

In summary, since the World Health Organisation's (WHO) groundbreaking definition in 1948, the concept of health has gained clarity and become increasingly widespread in scientific discussion. However, it has not yet become the leading reference category. When assessing the physical, mental and social well-being of individuals or population groups, the term 'illness' is still predominantly used, particularly in German-speaking countries.

Even today, the concept of health has not been firmly established in its own right. Abel argues that its 'powerlessness' will persist as long as it is defined in the context of medicine's capacity to define and control. Illness (or the absence thereof) remains the dominant reference point, with medicine as the leading science. A true balance between the two concepts has yet to be achieved in research, politics or public health practice. Consequently, Abel asserts that health is 'weakly and controversially defined, and unsuitable as a guiding paradigm' (2021, p. 35). The focus on illness reinforces established power structures in science, prevention, care, and health politics, favouring biomedical paradigms and structural safeguards. Abel explicitly praises the interdisciplinary and transdisciplinary approaches and definitional principles of Hurrelmann, Huber and Bircher (see above) because they do dispense with the concept of illness. Here, health is understood and operationalised as an active, variable, and multimodal process of adapting to and engaging with living conditions.

In the future, it will be important to develop the concept of health as a robust reference category. This will provide 'the prerequisite for gaining a better understanding of well-being, quality of life, and people's opportunities to realise their potential. Health is a multidisciplinary and interparadigmatic pursuit in itself, as the primary determinants of health are social living conditions‘ (2021, p. 36).

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All quotes from English sources were taken from the original literature, whereas we translated the German quotes. No substantial changes were made to the structure, text or intent of the German BIÖG Online Glossary article during translation. Wherever possible, we replaced German-only publications in the list of references with equivalent English publications by the same authors.

P.F. & K.H., September 2025

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